

Management of First Trimester Miscarriage Protocol (CG565)

Approval

Approval Group	Job Title, Chair of Committee	Date
Gynaecology Clinical Governance	Chair, Gynaecology Clinical Governance	20 th September 2024

Change History

Version	Date	Author, job title	Reason
4.0	June 2020	A Chase Consultant Gynaecologist	Reviewed and amended to reflect current practice and clinic information
5.0	August 2022	A Chase Consultant Gynaecologist C Prentice Consultant Obs & Gynae	Reviewed, minor changes pg2, 4 (5.2 criteria updated) & 5 (reference to GL1110 included and 5.4.2 criteria amended)
6.0	July 2024	A Chase, Consultant Gynaecologist C Prentice, Consultant O&G	Biennial review – no change to content, references updated on pg 13/14 RCOG CG154 replaced by NICE NG126

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Other relevant corporate or procedural documents:

This document should be read in conjunction with:

- [Anti-D guideline \(GL786\)](#)
- [Ectopic Pregnancy medical management protocol \(CG623\) & guideline \(GL1061\)](#)
- [Medical Management of Miscarriage protocol \(CG621\)](#)
- [MVA protocol \(CG700\)](#)
- [Pregnancy of Unknown Location guideline \(GL110\)](#)
- [Progesterone for Bleeding in Early Pregnancy CG758](#)

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1.0 Purpose

To assist medical and nursing staff in the general management of miscarriage. In particular, expectant, medical and surgical management.

Many complaints come from poor communication and contradictory advice.

The protocol aims to minimise this by standardising the information, advice and treatment that we provide to those couples who suffer early pregnancy loss.

2.0 Scope

For all staff, medical, nursing and clerical, to provide uniformity in the management of patients diagnosed with a first trimester miscarriage.

3.0 Roles and Responsibilities

The team consists of:

- Lead consultant
- Lead Sister and Senior Staff Nurses
- Lead Sonographer

4.0 Definitions

Early pregnancy loss (First trimester miscarriage) relates to the loss of a pregnancy up to 12+6 weeks from the last menstrual period.

5.0 Document content

5.1 Introduction

Miscarriage is the most common complication in early pregnancy and the incidence is as high as 31%. When considering clinically recognised pregnancies it occurs in 10–20% of pregnancies and can lead to significant psychological distress. Poor communication during this time is a recurring cause of complaint from women experiencing miscarriage. All patients must be counselled sensitively, with empathy and patience. The correct terminology should be used when counselling and for documentation. The following topics must be covered during counselling, and should be documented in the notes:

- Information on diagnosis and follow-up
- Miscarriage information leaflet given to the patient
- The patient and partner/companion have no further questions
- When and how to seek help if symptoms worsen or change

The Early Pregnancy and Acute Gynaecology Unit (EPAGU) has experienced nurses who can provide additional counseling if necessary.

The use of ultrasound (US) is key to nearly all early-pregnancy diagnoses.

Middle-grade doctors and trainees must demonstrate competency and be signed off by the Maternity Ultrasound Department before independently scanning.

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5.2 Viable Intrauterine Pregnancy (threatened miscarriage)

If the ultrasound shows a viable intrauterine pregnancy:

- Reassure the patient – Despite bleeding, 90% of pregnancies at 8 weeks will not miscarry
- If they have bleeding **and** a history of previous miscarriage consider offering progesterone (see Progesterone for Bleeding in Early Pregnancy CG758).
- Discharge back to the care of her GP
- Advise patients to contact the EPAGU +/- return to the clinic if:
 - Bleeding very heavily (Clots the size of their palm or soaking sanitary pads every 20 minutes)
 - Bleeding persists for more than 14 days (may need a repeat US)
 - Pain not controlled with simple analgesia

5.3 Pregnancy of Uncertain Viability

This has occurred when the ultrasound shows:

- Empty intrauterine gestation sac <25mm
- Fetal pole seen ≤7mm and no fetal heart beat seen

When this occurs:

- Plan for rescan in 7 - 14 days (minimum of 14 days if transabdominal US)
- Advise patients to contact the EPAGU number +/- return to the clinic if:
 - Bleeding very heavily (Clots the size of their palm or soaking sanitary pads every 20 minutes)
 - Pain not controlled with simple analgesia
 - Concerned/worried

5.4 Complete miscarriage

A strong history (passing clots, central cramping pain) is suggestive, but is not enough to confirm the diagnosis of complete miscarriage.

5.4.1 Diagnosing Complete Miscarriage

Complete miscarriage is likely when the ultrasound shows:

- Empty uterine cavity
- Tissue diameter (or endometrial thickness) <15mm (retained products are unlikely to be found on histology when <15mm).

If there is evidence of intrauterine pregnancy on an earlier ultrasound, complete miscarriage can be confirmed.

If there is no previous ultrasound in this pregnancy, ectopic pregnancy must be excluded. See PUL guideline (GL110) for protocol.

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A diagnosis of complete miscarriage is also suggested by products of conception seen by the attending gynaecologist (*and later confirmed by histology*).

5.4.2 Serum β HCG assessment

A fall in β HCG of **>50%** in 48 hours usually confirms complete miscarriage

A 48 hour **rise** in β HCG, or a **fall of <50%** indicates a pregnancy of unknown location (PUL). In this instance:

- Refer to PUL/Ectopic Pregnancy Protocol
- Discuss with consultant on call

Caution: Falling β HCG can occur in ectopic pregnancy and so careful attention to symptoms and history is needed.

5.4.3 After diagnosis of complete miscarriage.

Once complete miscarriage has been confidently diagnosed:

- Discharge patient to the care of her GP
- Advise patient to perform a home urine pregnancy test after 3 weeks.
- Advise patients to call the EPAGU if:
 - If the home urine pregnancy test taken after 3 weeks is positive
 - Bleeding persists for more than 2 weeks
 - Bleeding very heavily (clots the size of their palm or soaking sanitary pads every 20 minutes)
 - Pain not controlled with simple analgesia
 - Concerned/worried

5.5 Incomplete Miscarriage

This should be suspected when either:

- Examination findings: open internal os +/- POC in situ

OR

- Ultrasound findings: Intrauterine tissue diameter 15 - 50mm with no visible intrauterine sac

Management options:

1. Expectant (conservative) management is highly effective

- 80% resolve within 3 days
- Should be recommended as the first-line treatment for to all patients, unless specific Surgical Management of Miscarriage indications are present (see below)
- Follow up should be arranged as follows:
 - If the bleeding continues for 2-3 weeks, advise return for repeat ultrasound. They may then consider continuation of expectant management or surgical evacuation

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- If the bleeding stops, the patient should perform a home urine pregnancy test 3 weeks later and be discharged if negative

2. Surgical Management of Miscarriage (SMM), previously known as ERPC.

- Should be offered rather than recommended
- See detail in SMM section (below)

Note - Medical Management of incomplete miscarriage.

Medical management can be offered for incomplete miscarriage in the first trimester, although evidence suggests there is no advantage over expectant management (no difference in time to resolution).

This does not mean it is contraindicated. Some women will have a strong preference for medical management. Under these circumstances discuss with the consultant on call for possible medical management. A single dose of 800 micrograms of misoprostol can be used.

Second trimester miscarriage (>13 weeks) management is described in a separate document.

5.6 Missed Miscarriage

Absent miscarriage symptoms with US findings:

- Empty gestation sac >25mm (anembryonic gestation).

OR

- No embryonic heart activity with CRL>7mm (= Fetal loss).

OR

- No change / suboptimal increase in size of sac or CRL since previous scan 1w ago in case of GS <25mm or CRL <7mm

In this event, the patient should be given *the option* of having a confirmation scan by a second trained sonographer/doctor as soon as possible, ideally on the same day.

There are three management options:

A. **Expectant management** should be offered as first line management strategy.

Explore other management options as first line if;

- is at increased risk of haemorrhage (for example, she is in the late first trimester) **or**
 - has previous adverse and/or traumatic experience associated with pregnancy (for example, stillbirth, miscarriage or antepartum haemorrhage) **or**
 - at increased risk from the effects of haemorrhage (for example, if she has coagulopathies or is unable to have a blood transfusion) or there is evidence of infection.
- The patient should be advised:
- This option avoids surgical/ anaesthetic risks

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- Commencement of symptoms is unpredictable
- 90% will start within 3 weeks of diagnosis

B. Medical management

- The patient should be advised:
 - This option avoids surgical/anaesthetic risks.
 - Fewer emergency admissions than expectant management
 - Fewer 20% result in SMM (30% in expectant management)
 - Faster, more predictable commencement of bleeding than expectant management
 - No difference in duration of bleeding cf. expectant management
- See detail in Medical Management section (below) and [Medical Management of Miscarriage Protocol CG621](#).

C. Surgical Management of Miscarriage (SMM) – done under GA and also using Manual Vacuum Aspiration (local anaesthetic) for a selected group

- The patient should be advised:
 - Provides certainty of time to resolution (for most)
 - See detail in SMM section (below).
 - See detail in MVA protocol

5.7 Further Information on Modes of Management

There is no difference in infection rates (1-3%) between any of the modes of management for miscarriage.

5.7.1 Expectant (Conservative) Management of Miscarriage

The primary advantage of allowing the miscarriage to resolve ‘naturally’ is the avoidance of surgical and anaesthetic risks.

Approximately 30% (20% for medical management) of women will require surgical evacuation following expectant management of miscarriage.

Expectant management is by far the most cost effective mode of management.

If she is late in the first trimester SMM should be explored as may be preferable to expectant management after 10 weeks gestation (ultrasound dates rather than menstrual dates) or a tissue diameter >50mm, as pain and bleeding may be severe

5.7.1.1 Counselling for expectant management.

Well-informed patients experience less anxiety and are less likely to be readmitted. Advice on analgesia at home and what to expect during the miscarriage process aid this. It is helpful to give a broad indication of the likely degree of pain and blood loss:

- Very strong period-like pain.
- Heavy bleeding with blood clots requiring extra strength sanitary pads.

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- Pregnancy tissue, sac or fetus may be seen (give leaflet 'Sensitive Management of Pregnancy Tissue' - [sensitive-management-of-pregnancy-tissue.pdf](https://royalberkshire.nhs.uk/sensitive-management-of-pregnancy-tissue.pdf) (royalberkshire.nhs.uk))

5.7.1.2 Follow-up

Time to completion of the miscarriage process is unpredictable. Expectant management may be continued for as long as the patient is willing provided there are no signs of infection or excessive blood-loss.

Routine follow-up for missed miscarriage is 2-3 weeks later.

For complete or incomplete miscarriage:

- Discharge patient if no further bleeding and negative home urine pregnancy test after 3 weeks
- If continued bleeding, re-scan and re-consider management options.
- In general, review interval should be 2-3 weeks later.

Advise patients to call the EPAGU number if:

- Bleeding very heavily (Clots the size of their palm or soaking sanitary pads every 20 minutes)
- Pain not controlled with simple analgesia
- Concerned/worried

Patients do not require a repeat referral from their GP.

5.7.2 Medical Management of Miscarriage

Please see [Medical Management of First Trimester Miscarriage Protocol \(CG621\)](#)

Medical management is likely to be most useful when used for women with missed miscarriage.

Approximately 20% (30% for expectant management) of women will require surgical evacuation following medical management of miscarriage.

Medical management is less likely to be successful after failed expectant management.

It is preferable to SMM for second trimester miscarriage.

5.7.2.1 Off-label prescribing of Misoprostol

The doctor must document that the patient consents to the use of misoprostol, which is not licensed (off-label) for medical management of miscarriage. This is because it is sold for a different indication (gastric ulcers).

- Explain that off-label treatments are fairly common (most medicines administered to children are off-label) in medicine
- Explain that misoprostol is effective and safe in selected patients and is recommended by NICE.

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5.7.3 Surgical Management of Miscarriage (SMM)

SMM, previously known as ERPC, should be an option offered (rather than recommended) to women with incomplete or missed miscarriage.

The only advantages over expectant management:

- Predicable start and end point
- No follow up required

Note: Where infection is suspected, delaying surgical intervention for 12-24 hours is recommended to allow intravenous antibiotic (Augmentin) administration.

5.7.3.1 Indications for SMM

- Patient preference
- Persistent excessive bleeding/pain
- Haemodynamically unstable
- US tissue diameter of > 50mm (increased risk of excessive bleeding)
- US dates >10 weeks gestation (increased risk of excessive bleeding)
- Evidence of infected retained tissue (proceed after 6-24h IV antibiotics)
- Suspected gestational trophoblastic disease
- Failed medical/ surgical management

5.7.3.2 Pre-op protocol

- Complete the EPAGU SMM pathway at time of consultation
- Provide SMM patient information leaflet
- Book as an elective day case on a future List (unless emergency indication)
- Patients requiring emergency SMM should be added to the Emergency theatre list
- MRSA swabs, FBC, Rhesus group & save (Needs to be valid within 3 days), +/-
- Procedures other than SMM for miscarriage must be discussed with the consultant. These include repeat SMMs, molar pregnancies, retained products of conception and laparoscopies.

5.7.3.3 Consent for SMM

In addition to SMM consent, patients should also consent to:

- Laboratory analysis to confirm the diagnosis and exclude molar pregnancy
- Consent form for Sensitive Management of Pregnancy Tissue should be completed and signed by the patient. Tissue sent to histology will not be processed without the pink copy of this consent form.

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- Mortuary blessing and cremation. Patients have the right to keep their products of conception (refer to RBH sensitive disposal of early pregnancy remains pathway)
- Swabs for Chlamydia and gonorrhea when indicated

SMM complications include:

- Haemorrhage (<3%)
- Infection (2-3%) - no increase over expectant /medical management.
- Incomplete evacuation requiring possible further ERPC <1%
- Uterine perforation (<1%), cervical tears
- Bowel / bladder trauma (<1%)
- Intrauterine adhesions (<5%)
- No additional risk to fertility

Pre-procedure

Scan

On the day of surgery, If the patient has had significant bleeding since their last scan, a rescan would be advised to ensure that surgery is still necessary.

5.7.3.4 Cervical preparation.

Pre-op cervical preparation is indicated for missed and most incomplete miscarriages (closed internal os on bimanual examination). It is not indicated in patients with incomplete miscarriage and an open internal cervical os (assessed on initial bimanual examination).

Pre-op cervical preparation results in reduced dilatation force, haemorrhage and uterine/cervical trauma.

Doses:

Misoprostol dose: 600µg PV (400µg for women with previous caesarean section) 1-2 hours pre-op.

5.7.3.5 Intra-op protocol

Women who are at increased risk (usually women under the age of 25) should either be screened for lower genital tract organisms prior the procedure and treatment of positive cases or be given prophylactic antibiotics at operation.

- Take endocervical swab during surgery and request "CT & GC NAAT + MC&S" on the form.
- These investigations require pre-op consent.

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Anti-D immunoglobulin should be administered to all non-sensitised Rh negative women in theatre (WHO checklist). Refer to Anti-D Prophylaxis [Anti-D guideline \(GL786\)](#).

Antibiotic prophylaxis:

Metronidazole 1g PR at time of operation and doxycycline 100mg BD for next 7 days. Allergic patient can be given an alternative.

5.7.3.6 Products of conception (POC) histology

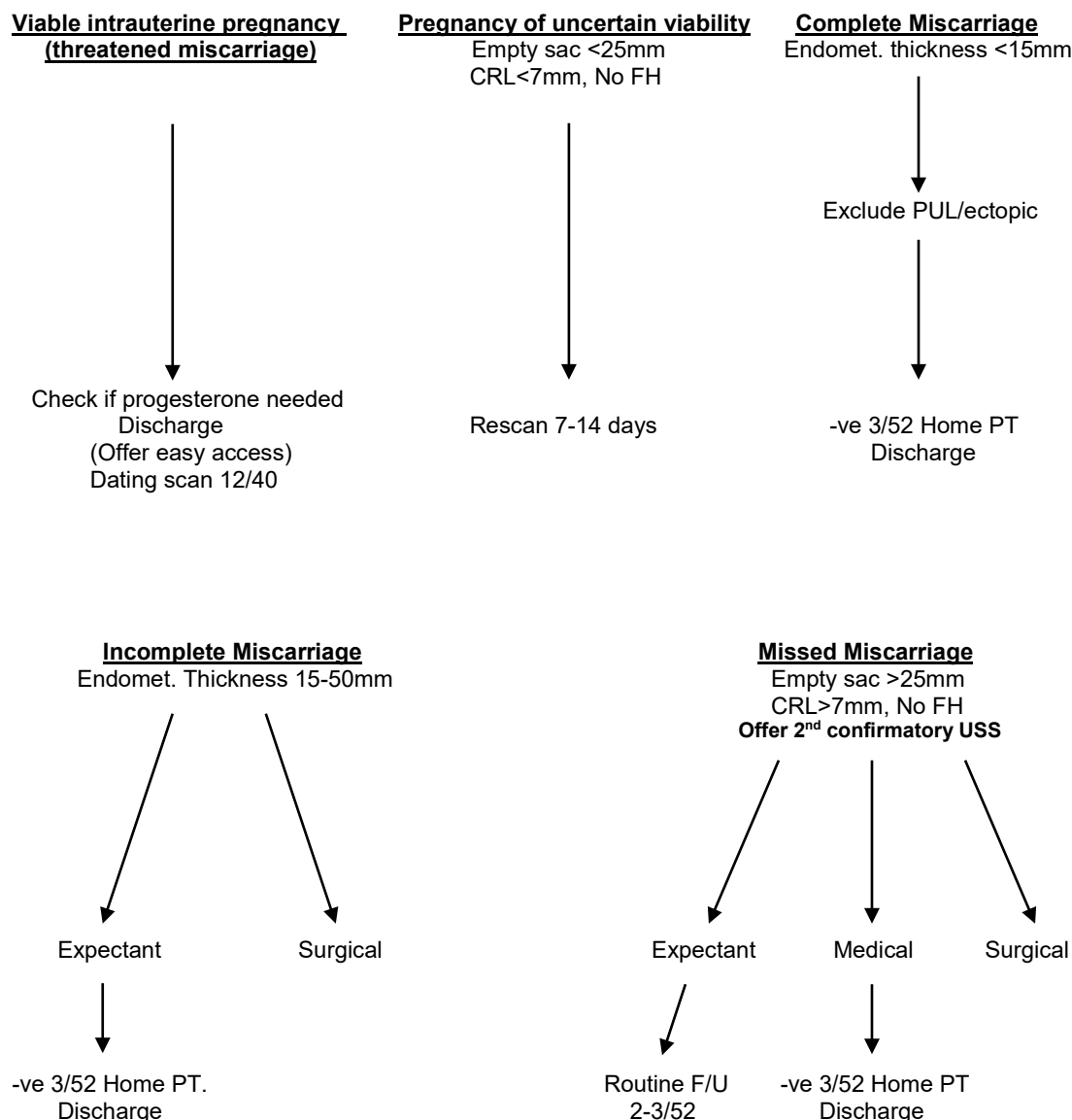
- Ensure that POC are seen and documented at evacuation.
- Send an adequate sample to histology to confirm intrauterine pregnancy and exclude molar pregnancy. If unsure, or if minimal sample is obtained, the surgeon should empty the suction trap and send the entire sample. The pink copy of the Sensitive Management of Pregnancy Tissue consent must be sent with any samples. Histology will not process without the pink copy.
- It is the surgeon's responsibility to chase the histology result of all POC samples. This is essential if minimal or no sample is obtained (risk of ectopic pregnancy), or there is a risk of molar pregnancy.
- If this miscarriage is the third consecutive miscarriage, offer for the POC to be sent to Oxford for cytogenetics, if this has not already been done in the past. You must add 'testing for cytogenetics' to the consent form. If POC are being sent for cytogenetics, they must not be sent in formalin and some POC must still be sent for histology.

5.7.3.7 Post-op protocol

- Give the EPAGU contact number for queries relating to potential post-op complications.
- Complete the electronic discharge letter.
- GP should provide routine follow-up care.

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5.8 Summary of Management Pathways



6.0 Consultation Undertaken

Emergency Gynaecology and Sonning Ward Sisters

Gynaecology Clinical Governance Committee – This includes: a patient representative, Gynaecology Operations Managers, Gynaecology consultants, Junior and middle grade medical staff.

A copy of the protocol was circulated to all stakeholders for amendments prior to a scheduled clinical governance meeting.

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7.0 Dissemination/Circulation/Archiving

- A hard copy will be kept in the EPAGU and on Sonning Ward.
- A hard copy will be given to all junior medical staff at induction.
- Copy will be e-mailed all medical staff.
- Protocol will be available on the Trust intranet (Gynaecology section) and external website.
- The Maternity Information Officer will be responsible for archiving old versions of this document.

8.0 Implementation

Medical and nursing staff will be reminded of protocol by the Emergency Gynaecology Sister and the Gynaecology consultants.

9.0 Training

There is no mandatory training associated with this procedure. If staff have queries about its operation, they should contact their line manager in the first instance.

10.0 Monitoring of Compliance

Aspect of compliance or effectiveness being monitored	Monitoring method	Individual or dept. responsible for the monitoring	Frequency of the monitoring activity	Group/committee which will receive the findings/ monitoring report	Committee/ individual responsible for ensuring that the actions are completed
Miscarriage management. Reasons for Complaints	Oversight of Juniors & Complaint reviews	Emergency Gynaecology	Monthly	Gynaecology MDT/Governance	W Kuteesa / J Waite

The Trust reserves the right to amend its monitoring requirements in order to meet the changing needs of the organisation.

11.0 Supporting Documentation and References

1. Association of Early Pregnancy Units. Guidelines 2007. Association of Early Pregnancy Units; 2007. <https://www.aepu.org.uk/>
2. Miscarriage Association. Management of Miscarriage: surgical, medical, conservative. Wakefield: Miscarriage Association; 2010. <https://www.miscarriageassociation.org.uk/wp-content/uploads/2023/05/Management-of-miscarriage.pdf>

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3. National Institute for Health and Clinical Excellence (NICE). Ectopic pregnancy and miscarriage: Diagnosis and initial management in early pregnancy of ectopic pregnancy and miscarriage. Royal College of Obstetricians and Gynaecologists. London: RCOG; (updated August 2023) <https://www.nice.org.uk/guidance/ng126>
4. Trinder J, Brocklehurst P, Porter R, Read M, Vyas S, Smith L. Management of miscarriage: expectant, medical, or surgical? Results of randomised controlled trial (miscarriage treatment (MIST) trial). *BMJ* 2006;332:1235–40.
5. Sagili H, Divers M. Modern management of miscarriage. *The Obstetrician & Gynaecologist* 2007;9:102–108.
6. Sagili H, Mohamed K. Pregnancy of unknown location: an evidence-based approach to management. *The Obstetrician & Gynaecologist* 2008;10:224–230.

12.0 Equality Impact Assessment

Stage 1: Screening

Part 1: Initial Scoping

For each of the nine protected groups identified in the table below, respond to the identified questions with a Yes (Y); No (N); or Unclear (U)

	Age	Sex	Disability	Race	Gender Reassignment	Religion or Belief	Sexual Orientation	Marriage and Civil Partnership	Pregnancy and Maternity
Do different groups have different needs, experiences, issues and priorities in relation to the proposed policy/change proposal?	N	Y	N	N	Y	N	N	N	Y
Is there potential for or evidence that the proposed policy/change will not promote equality of opportunity for all and promote good relations between different groups?	N	N	N	N	N	N	N	N	N
Is there potential for or evidence that the proposed policy will affect different population groups differently (including unintended discrimination against certain groups)?	N	Y	N	N	Y	N	N	N	Y
Is there public concern (including media, academic, voluntary or sector specific interest) in potential discrimination against a particular population group or groups?	N	N	N	N	N	N	N	N	N

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Part 2: Evidence and Feedback that has informed your analysis

Please identify below the data, information or feedback that you have drawn on to reach the conclusions above. This will be information that has enabled you to assess the actual or potential impacts in the context of the key needs to **eliminate unlawful discrimination, advance equality of opportunity** and **foster good relations** with respect to the characteristics protected by equality law. These sources could include:

- Equalities monitoring information of staff/service users affected by the identified provision/policy etc.
- Engagement (internal/external or both) with or feedback from relevant stakeholders e.g. staff; patient groups, commissioners, external agencies.
- Staff Survey Data; Patient Survey Data etc.
- Research or information available relative to the identified protected group.
- Project leads professional knowledge of the issues the policy/change is seeking to enact.

N/A

If the analysis under Part 1 has concluded that there are equality impacts or that the impacts are unclear (i.e. you responded 'Yes' or 'Unclear' in Part 1), **please move on to Part 4 of the assessment**. If no equality impacts are identified, **please move on to Part 3 below** to conclude the assessment

Part 3: Narrative

If you have concluded there are no equality impacts related to the policy/provision, please provide a brief narrative to explain why you have come to this conclusion:

N/A

If no equality impacts have been identified, this concludes the equality impact assessment. Please complete the declaration below:

Based on the information set out above I have decided that a full equality impact assessment is (please delete as appropriate):

~~Necessary~~ / **NOT necessary.**

Stage 2: Full Equality Impact Assessment

Part 4: Identifying the Potential Impacts

Is there concern that the policy/strategy etc. will not promote equality of opportunity, eliminate discrimination or promote good relations?

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Looking back at part one of the EIA, please summarise those areas where there are concerns that the policy/strategy etc. could have negative impacts and why this is the case. Consider all equality areas where a potential negative impact has been identified.

Protocol only affects genetic pregnant females.

What gaps are there (if any) in the information required to identify impacts? How will this be redressed?

N/A

Part 5: Previous or Planned Consultation

Using the table below, give a summary of what previous or planned consultation on this policy etc **has or will** take place with groups or individuals from the equality groups and what has this consultation noted about the likely negative impact?

Equality Group/ Representative Body	Summary of consultation and notes of likely negative impacts

What consultation has taken place or is planned with Trust staff including staff that have or will have direct experience of implementing the strategy, policy or practice?

Discussed at MDT CG meetings.

Part 6: Managing and minimising potential negative impacts

If negative/potential negative impacts have been identified, you should identify in this section how the issue/problem will be addressed or mitigated and who will be responsible for addressing it.

Issue	Action/Mitigation	Timescale	Responsible Person

What monitoring and evaluation process is in place to check, review and change (as and if required) the implementation of this policy from an equalities perspective?

CG complaints procedures.

This concludes the Equality Impact Assessment

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